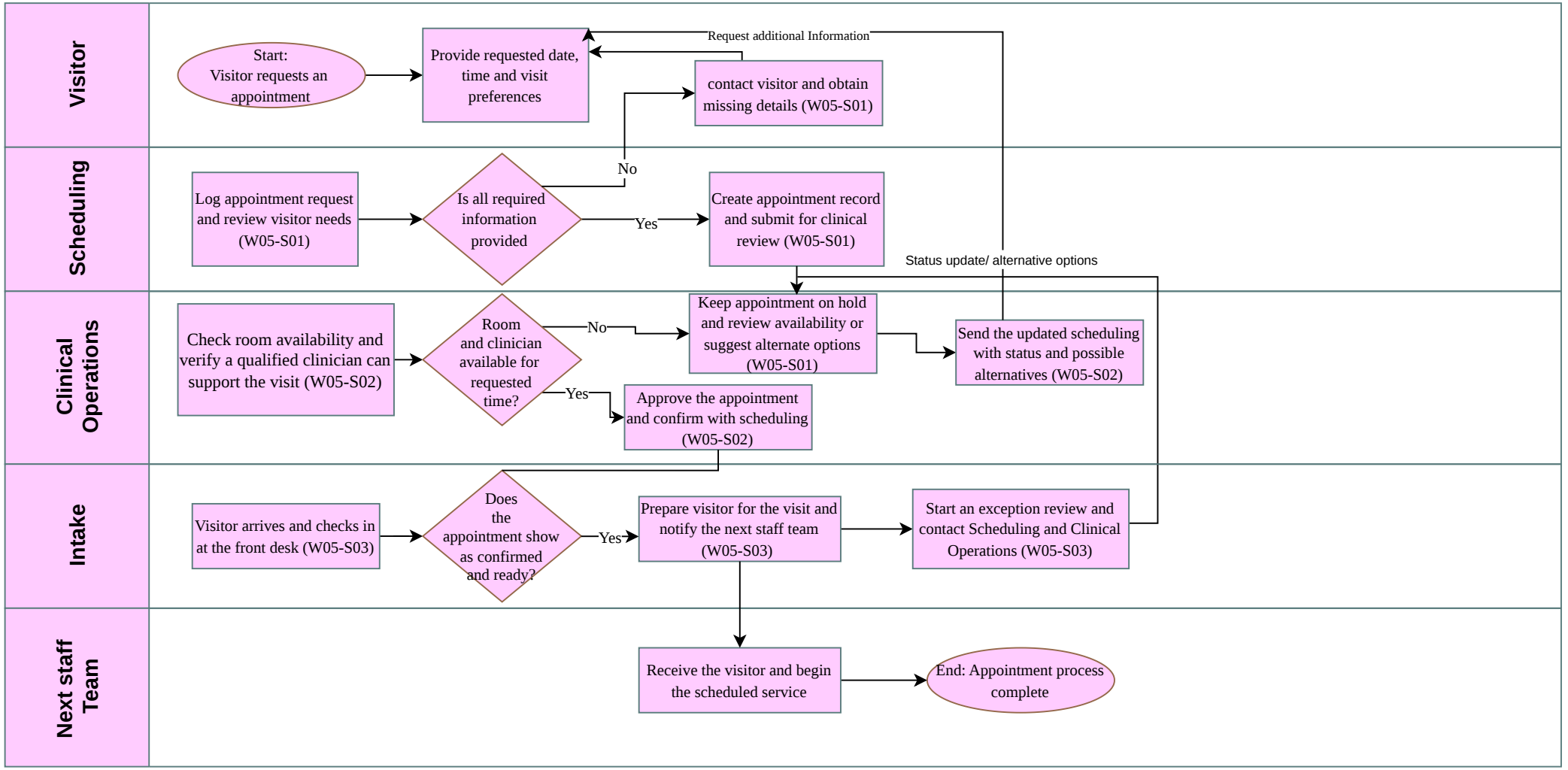
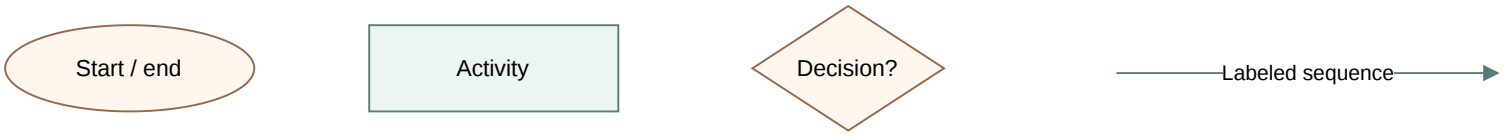
CIT 140 · Week 5 | Current Appointment Process

Replace this line with your process start and end. Add supported steps and decisions in the responsible role's lane. Label decision branches and source IDs.



Symbol key (copy these shapes into your lanes; they are not case steps)



Week 5 | Scope and Findings

1 · Process scope
Name: Desiree Kamerer Fuentes;
Purpose: Track an appointment from the visitor's request through scheduling, readiness review, intake, and handoff
Start: Visitor requests an appointment.
End: Intake confirms readiness and hands the visitor to the next staff team.
Roles: Visitor, scheduling, Clinical Operations, Intake, Next Staff Team.
Outside scope: Software redesign and clinical decisions.

2 · Check the two paths
Ordinary Path: W05-S01 shows Scheduling recording appointment details and requesting a readiness decision. W05-S02 shows Clinical Operations checking room and clinician availability. W05-S03 shows Intake checking status, confirming support arrangements, and completing the handoff.
Incident Path: W05-S03 records a booking confirmation and visitor arrival, bit intake still displayed the appointment as held. Dana opened an exception review.
Gap: The trace does not show a completed readiness decision reaching intake before arrival. It does not establish why the decision was missing.

3 · Evidence and model review
Sources: W05-S01 supports scheduling; W05-S02 supports readiness checks; W05-S03 supports intake and handoff; W05-S04 supports the incident trace.
Review: The diagram separates booking confirmation from readiness and shows the held and exception paths.
Uncertainty: The records do not explain why readiness was missing. Scheduling and Clinical Operations must review the event history to clarify the gap.

4 · Learning reflection (reuse your Discord paragraph)
This assignment helped me understand how an appointment moves through scheduling, Clinical Operations, and Intake. I learned that receiving a booking confirmation does not necessarily mean the appointment is ready, because the room and a qualified clinician must also be available. The incident record showed that Intake still displayed the appointment as held when the visitor arrived, but it did not explain why the readiness decision was missing from the available trace. This showed me how important clear communication, accurate records, and responsibility during handoffs are. In week 4, I focused on what systems need to do, while in week 5, I used the clinical records to map how the current process actually works and identify where information becomes unclear.

5 · Portfolio evidence
Editable File:Fuentes_Desiree_CIT140_Week05_As_Is_Process_Model.drawio
PDF: Fuentes_Desiree_CIT140_Week05_As_Is_Process_Model.drawio.pdf
Screenshot: Week05_Discord_Post.png
Receipt: Week05_Mission_Receipt.pdf